

Approved Drug Formulary

Dosing, Storage, Tier and Substitution Reference

MediAssist Health Network

Pharmacy & Therapeutics Committee

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Access: Doctors and Admin

The MediAssist Approved Drug Formulary lists medicines approved for use across all hospitals, with standard dosing, storage, formulary tier and substitution rules. Prescribers must select from this formulary; out-of-formulary requests follow the process in the final section.

Formulary Tier System

Every medicine in this formulary carries a tier that governs how it is authorised:

- **Tier 1** — generic first-line; open to all prescribers and subsidised for staff.
- **Tier 2** — branded preferred; open to prescribers.
- **Tier 3** — specialist use; requires HOD approval.
- **Tier 4** — restricted; requires CMO approval.

1. Antimicrobials

Drug	Class	Route	Standard Dose	Tier	Storage	Notes
Amoxicillin	Penicillin	Oral	500 mg TDS	1	Room temp	Avoid in penicillin allergy
Amoxicillin-Clavulanate	Penicillin/inhibitor	Oral/IV	1.2 g Q8H	2	Room temp	GI upset common
Piperacillin-Tazobactam	Beta-lactam/inhibitor	IV	4.5 g Q8H	2	Refrigerate post-recon.	De-escalate on culture
Ceftriaxone	3rd-gen cephalosporin	IV/IM	1–2 g OD	2	Room temp	Avoid with calcium in neonates
Azithromycin	Macrolide	Oral/IV	500 mg OD	1	Room temp	QTc prolongation risk
Clarithromycin	Macrolide	Oral	500 mg BD	2	Room temp	Many drug interactions
Meropenem	Carbapenem	IV	1 g Q8H	3	Room temp	HOD approval required
Vancomycin	Glycopeptide	IV	15–20 mg/kg Q12H	3	Room temp	TDM; trough 15–20 mg/L
Linezolid	Oxazolidinone	Oral/IV	600 mg BD	3	Room temp	Monitor platelets if > 14 days
Metronidazole	Nitroimidazole	Oral/IV	500 mg TDS	1	Room temp	Avoid alcohol
Ciprofloxacin	Fluoroquinolone	Oral/IV	500 mg BD	2	Room temp	Avoid in children < 18 yr
Clindamycin	Lincosamide	Oral/IV	600 mg Q8H	2	Room temp	C. difficile risk
Doxycycline	Tetracycline	Oral	100 mg BD	1	Room temp	Photosensitivity
Fluconazole	Azole antifungal	Oral/IV	150–400 mg OD	2	Room temp	Hepatic monitoring
Acyclovir	Antiviral	Oral/IV	5–10 mg/kg Q8H	2	Room temp	Maintain hydration
Colistin	Polymyxin	IV	Per ideal body weight	4	Room temp	CMO approval; nephrotoxic

2. Cardiovascular Drugs

Drug	Route	Standard Dose	Tier	Contraindications
Amlodipine	Oral	5–10 mg OD	1	Severe aortic stenosis
Telmisartan	Oral	40–80 mg OD	1	Pregnancy; bilateral renal artery stenosis
Atorvastatin	Oral	10–80 mg nocte	1	Active liver disease
Metoprolol	Oral/IV	25–100 mg BD	1	Cardiogenic shock; 2°/3° heart block
Ramipril	Oral	2.5–10 mg OD	1	Pregnancy; angioedema history
Aspirin	Oral	75–300 mg OD	1	Active peptic ulcer; bleeding
Clopidogrel	Oral	75 mg OD (300 mg load)	2	Active bleeding
Digoxin	Oral/IV	0.125–0.25 mg OD	2	VF; toxicity if hypokalaemic
Furosemide	Oral/IV	20–80 mg OD/BD	1	Anuria; severe hypokalaemia
Spironolactone	Oral	25–50 mg OD	2	Hyperkalaemia; severe renal impairment

3. Analgesics & Antipyretics

Drug	Route(s)	Dose Range	Controlled?
Paracetamol	Oral / IV / Suppository	0.5–1 g Q6H (max 4 g/day)	No
Ibuprofen	Oral	200–400 mg Q8H	No
Diclofenac	Oral / IM	50 mg BD–TDS	No
Tramadol	Oral / IV	50–100 mg Q6H	Yes (schedule)
Morphine	Oral / IV / SC	Titrated to effect	Yes — narcotic register
Fentanyl	IV / transdermal	Titrated; patch 12–100 µg/h	Yes — narcotic register
Buprenorphine	Sublingual / patch	Per pain protocol	Yes (schedule)
Pregabalin	Oral	75–150 mg BD	Yes (schedule)

4. Respiratory Drugs

Drug	Route	Standard Dose	Tier	Notes
Salbutamol	Inhaled / Neb	100–200 µg PRN / 2.5–5 mg neb	1	Tremor, tachycardia
Ipratropium	Inhaled / Neb	500 µg Q6H neb	1	Dry mouth
Budesonide	Inhaled	200–400 µg BD	2	Rinse mouth; candidiasis
Montelukast	Oral	10 mg nocte	2	Neuropsychiatric caution
Theophylline	Oral	Per level	3	Narrow therapeutic index; TDM

5. Gastrointestinal & Endocrine Drugs

Drug	Route	Standard Dose	Tier	Notes
Pantoprazole	Oral / IV	40 mg OD	1	PPI; review long-term use
Ondansetron	Oral / IV	4–8 mg Q8H	1	QTc caution
Metoclopramide	Oral / IV	10 mg TDS	1	Avoid prolonged use (EPS)
Insulin (regular)	SC / IV	Per sliding scale	2	High-alert; double-check
Levothyroxine	Oral	25–150 µg OD	1	Empty stomach; recheck TSH 6-weekly
Hydrocortisone	IV	100 mg Q6–8H	2	Stress dosing; monitor glucose

6. Renal Dose Adjustment (selected drugs)

Doses below are illustrative; confirm against the latest renal dosing reference and the patient's eGFR before prescribing.

Drug	eGFR 30–50	eGFR < 30
Metformin	Max 1 g/day; review	Avoid
Meropenem	Reduce frequency to Q12H	Q24H; specialist input
Vancomycin	Extend interval per TDM	Per TDM; close monitoring
Enoxaparin	Standard with monitoring	Reduce to OD; consider UFH
Digoxin	Reduce dose; check levels	Marked reduction; toxicity risk

7. High-Alert Medications

High-alert

The medicines below carry a heightened risk of significant patient harm when used in error. They require an **independent double-check by two staff** before administration and are restricted to trained personnel.

Drug	Special Storage	Key Safeguard
Insulin	Refrigerate 2–8 °C	Units never abbreviated as 'U'; double-check dose
Heparin	Room temp; separate from insulin	Double-check; aPTT monitoring
Potassium Chloride (conc.)	Locked; never on open shelf	Never IV push; dilute and infuse
Morphine	Narcotic cupboard, double-locked	Narcotic register entry per dose
Methotrexate	Room temp; cytotoxic labelling	Weekly (not daily) dosing; confirm schedule

8. Substitution Policy

- When a **Tier 2** drug is unavailable, the pharmacist may substitute the equivalent **Tier 1** generic **without** a prescription change.
- **Tier 3 / Tier 4** substitutions require the prescribing doctor's written approval.
- All substitutions are logged in the pharmacy system and visible on the patient record.

9. Out-of-Formulary Request Process

When a required medicine is not on the formulary, follow these steps:

- 1 The prescriber submits an out-of-formulary (OOF) request form with clinical justification.
- 2 The Pharmacy & Therapeutics committee reviews the request within 72 hours.
- 3 For urgent or life-saving cases, the CMO may approve the same day.

10. Look-Alike, Sound-Alike (LASA) Medications

Dispensing safety

LASA pairs are a leading cause of dispensing error. Confirm the full drug name, strength and patient before dispensing; 'Tall Man' lettering is used on labels.

Drug A	Confused with	Distinguishing note
DOPamine	DOBUamine	Different haemodynamic effect; confirm infusion label
epHEdrine	epINEPHrine	Very different potency; check ampoule
ceFAZolin	cefTRIAxone	Confirm spectrum and indication
hydrOXYzine	hydrALAZINE	Antihistamine vs antihypertensive
clonazePAM	cloNIDine	Anticonvulsant vs antihypertensive

11. IV Fluids Quick Reference

Fluid	Tonicity	Typical Use
0.9% Sodium Chloride	Isotonic	Resuscitation, maintenance
Ringer's Lactate	Isotonic (balanced)	Resuscitation; avoid in hyperkalaemia
5% Dextrose	Isotonic→hypotonic	Free water; hypernatraemia
DNS (5% Dext + 0.9% NaCl)	Hypertonic	Maintenance with glucose
20% Mannitol	Hypertonic	Raised intracranial pressure (specialist)

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